A/PROF CHAMARA BASNAYAKE
MBBS, PhD, FRACP
Gastroenterologist & Endoscopist
PN 421926PK

FOCUS GASTROENTEROLOGY

15 April 2026

Dr Louise Huxley
Ochre Medical Centre

Dear Dr Huxley,

Re: Ms Zyla Williams
DOB: 2006-08-01
Address: 15 Salier Crescent, MOUNT STUART TAS 7000

Summary

- **Deterioration of gastrointestinal symptoms following the cessation of nortriptyline.**
- **Chronic constipation and persistent abdominal distension.**
- **Recent haemorrhoidal bleeding, now resolved, with ongoing local swelling.**
- **Trial of rifaximin and continuation of high-dose famotidine.**
- **Action required: Please consider uptitrating sertraline to 100–150 mg if gastrointestinal symptoms remain refractory after the rifaximin trial.**

I had the pleasure of reviewing Zyla to discuss her ongoing gastrointestinal symptoms. Unfortunately, her clinical state has deteriorated recently, characterised by significant abdominal distension and subcostal pain. I suspect the subcostal discomfort is reflux-related. These symptoms appear to have worsened following the cessation of nortriptyline, which was discontinued due to concerns that interactions with sertraline and Vyvanse were exacerbating her obsessive-compulsive disorder.

Regarding her bowel habit, Zyla is experiencing significant constipation. She is currently using Osmolax twice daily but continues to have infrequent bowel movements. She also reported a recent episode of haematochezia and perianal swelling associated with a haemorrhoid. While the bleeding has resolved following a one-week course of topical treatment, she feels the area remains swollen. I have advised her to extend the use of topical therapy, such as Proctosedyl, to a minimum of two weeks of twice-daily application followed by a further two weeks of daily application to ensure the inflammation settled. I note that previous faecal testing identified Blastocystis; however, I have

All appointments and enquires
Suite 201, 100 Victoria Parade
East Melbourne VIC 3002
Ph: 03 9650 7917
Fax: 03 9650 7910
Pager: 03 9387 1000
Email: office@focusgastro.com.au

Endoscopy locations
Epworth Freemasons Day Proc. Centre
320 Victoria Pde, East Melbourne, 3002
Hobson Healthcare Sydenham
566 Melton Hwy, Sydenham, 3037
St Vincent's Private Hospital
59 Victoria Pde, Fitzroy, 3065

advised Zyla that this is a common bystander and does not require antimicrobial treatment.

Furthermore, Zyla mentioned a suspected new dairy allergy, noting symptoms of dyspnoea and throat pressure after consumption, though she appears to tolerate lactose-free dairy. As these symptoms are atypical for simple lactose intolerance, I am pleased she has a pending review with an allergist for formal evaluation.

Impression and Plan

My impression is that the cessation of nortriptyline has removed a degree of control over her visceral sensation and gut motility. We discussed various pharmacological options, including restarting low-dose nortriptyline (5 mg) or trialling prucalopride. However, to avoid further psychiatric interactions, we have opted for a more conservative approach initially.

Zyla will commence a two-week course of rifaximin (500 mg three times daily), which she will source from a compounding pharmacy to manage the cost. This is intended to address her bloating and potentially assist with motility by modulating the gut microbiota. She will continue famotidine at a dose of 40 mg twice daily and maintain daily Osmolax. If her symptoms do not show significant improvement one month after completing the rifaximin, I have suggested that we increase her sertraline to 100 mg, and potentially 150 mg, as the prokinetic side effects of a higher dose may benefit her bowel habit. I would be grateful for your assistance in monitoring this titration should it be necessary. I will review Zyla again in three months.

Kind regards

Yours sincerely,

A/Prof Chamara Basnayake
Gastroenterologist